

# OSCE

## Skin

# Impetigo

- is a contagious superficial bacterial infection of skin
- **Classified as**
- **Primary** :-direct bacterial invasion of previously normal skin and
- **Secondary**:-infection at sites of minor skin trauma

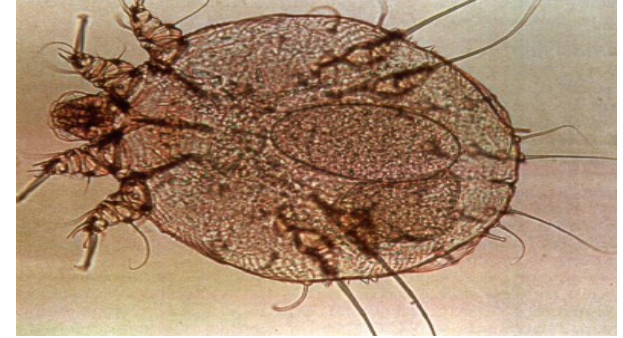


- Common in 3-5 year olds
- Most common cause is Strept and S.Aureus
- R/F-poverty, crowding, poor hygiene, and underlying scabies
- Variants are Non-bullous, Bullous, and Ecthyma.
- MX-Topical therapy with mupirocin 2%, fusidic acid, and retapamulin 1% and systemic therapy with oral antibiotics.

# Impetigo Pictures



# Scabies

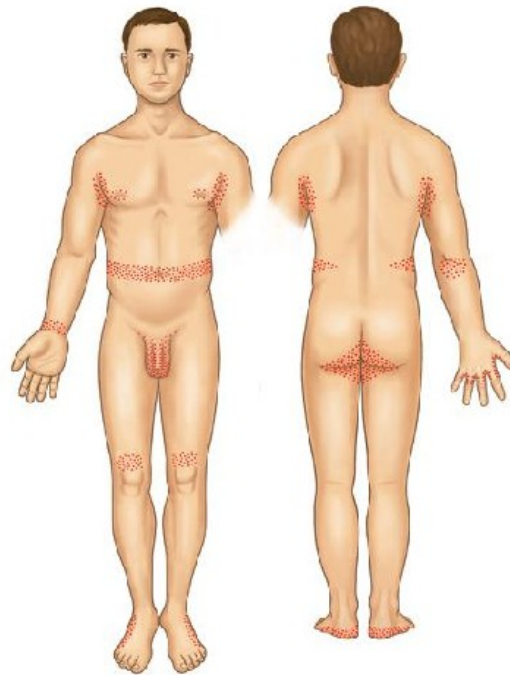


- *Sarcoptes scabiei*, the itch mite, is an oval, ventrally flattened mite with dorsal spines
- Itchiness is the most obvious symptom of scabies
- It is worse at night time when patient is warm
- The pathognomonic sign of scabies is a burrows appear as slightly elevated, grayish, tortuous lines in the skin.

- *Typical sites involvement (in adults):*
  - *interdigital webbing of the hands,*
  - *flexural aspect of the wrists, behind the ears, axilla, waist, ankles, feet, buttocks & belt area*
  - *penile & scrotal in men, areola, nipples & genital area in women*
  - *The circle of Hebra - an imaginary circle intersecting the main sites of invt – nipple, axillae, elbow flexures, wrists & hands, & crotch/groin*
  - *In infants, elderly & immunocompromised hosts, all skin surfaces are susceptible, including the scalp & face*

- Variants include nodular, bullous and norwegian/ hyperkeratotic/crusted

## Scabies distribution



Scabies usually involves the sides and webs of the fingers, the flexor aspects of the wrists, the extensor aspects of the elbows, anterior and posterior axillary folds, the skin immediately adjacent to the nipples (especially in women), the periumbilical areas, waist, male genitalia (scrotum, penile shaft and glans), the extensor surface of the knees, the lower half of the buttocks and adjacent thighs, and the lateral and posterior aspects of the feet. The back is relatively free of involvement; the head is spared except in very young children.

### Scabies, infant



The rash of scabies may be more diffuse in infants, spreading to the trunk and scalp in addition to the extremities.





# IX and MX of scabies

## Diagnosis

- **Mainly clinical**

- *Pruritus with typical lesions & distribution*
- *Pruritus in household members or close personal contacts*

### **Microscope**

- *Scrap an unscratched burrow---- → cover with a drop of immersion oil, ----- → scrap with a No.15 blade or dermal curette & placed on a glass slide with a coverslip-----  
→ Under the **10x** objective of the microscope, **the female mite and/or her eggs or feces are seen***

- *skin scrapings obtained from the finger webs, wrists, or ankles is most likely to be positive*
- *In Norwegian scabies, scraping of the thick scales will often yield several viable mites*
- *Excoriated lesions are often negative*

# Management of scabies mainly ivermectin and permethrin

## TREATMENT OVERVIEW

- 5% permethrin cream: This is the most common treatment for scabies. It is safe for children as young as 1 month old and women who are pregnant.
- 25% benzyl benzoate lotion.
- 10% sulfur ointment.
- 10% crotamiton cream.
- 1% lindane lotion.
  
- Some patients need other treatment, too.
- Antihistamine: To control the itch and help you sleep.
- Pramoxine lotion: To control the itch.
- Antibiotic: To wipe out an infection.
- Steroid cream: To ease the redness, swelling, and itch.

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## Scabies Control

- Educate patient about personal hygiene, including hand washing
- Increase awareness and surveillance for scabies
- Instruct patient to bath thoroughly, scrubbing the involved areas with a brush.
- Educate other members of the family.
- All members of the family should be treated on the same day.

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# Molluscum Contagiosum



Several fleshy lesions with umbilicated centers on the face. They tend to regress and disappear after some months.



# Molluscum DDX

|               |                                                                                                                                                                      |
|---------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Infections    | Verruca vulgaris <sup>88</sup><br>Chickenpox <sup>89</sup><br>Folliculitis <sup>1</sup><br>Condyloma acuminatum <sup>1</sup>                                         |
| Tumors        | Syringomas <sup>90</sup><br>Steatocystoma multiplex <sup>1,90</sup><br>Basal cell carcinoma <sup>91</sup>                                                            |
| Miscellaneous | Acne vulgaris <sup>92</sup><br>Gianotti-Crosti Syndrome <sup>25</sup><br>Milia <sup>1</sup><br>Sebaceous hyperplasia <sup>93</sup><br>Papular urticaria <sup>1</sup> |

- Molluscum contagiosum is a benign disease caused by the poxvirus family and spread by close human contact.
- The lesions are characteristic firm white nodules which vary greatly in number and tend to persist for a period of a few weeks to a few months.
- Sometimes they may be found in clusters along a scar

# Wart



- Most common warts are 2-10mm in diameter (although they may coalesce to larger masses), flesh-coloured or brown, keratotic papules with a rough surface.

# Very important

## DD<sub>x</sub> of Viral Exanthems

### Erythematous eruptions

#### morbiliform

- Measles
- Rubella
- Erythema infectiosum
- Roseola infantum
- CMV
- Infectious Mononucleosis

### Vesiculopustular lesions

- HSV infection
- Varicella & Herpes zoster
- Kaposi's Varicelliform erup.

### Purpuric lesions

- Atypical measles
- Congenital rubella
- Congenital CMV

# Mongolian spot



- Blue- grey poorly circumscribed, multiple or single maculae found most commonly on the lumbosacral region